

## Summary of recommendations (Q1, Q2, Q8, Q9, Q10)

1. Require continuous evaluation for any federally supported AI implementation.  
Make evaluation a non-negotiable component of AI deployment, aligned with the Evidence Act's intent, with clear expectations for what is measured, how often, and how results are used to adjust or even pause implementation (Q1, Q2, Q8, Q10).
2. Build an evaluation framework that uses EHR and EHR-use metadata.  
Use existing EHR metadata (e.g., audit logs, documentation patterns, workflow traces) to measure how AI affects workflow, team dynamics, and the cognitive environment, rather than just whether the AI is on (Q1, Q2, Q8, Q10).<sup>1</sup>
3. Tie funding and incentives to demonstrated value, not just use  
Link grants, pilots, and payment adjustments to improvements in a small set of meaningful, feasible, and viable measures (for example, burden reduction, timeliness, care coordination), rather than to raw AI adoption rates (Q1, Q2, Q10).<sup>2</sup>
4. Focus on "automation of the obvious, flagging of the not-so-obvious."  
Encourage AI that automates routine, well-understood tasks and surfaces non-obvious risks, while preserving human expertise and judgment for complex and atypical cases (Q1, Q2, Q9, Q10).
5. Guard against automation complacency and overload.  
Require designs and training that keep humans actively engaged, monitor for over-reliance and new types of error, and ensure clinicians and staff can operate safely even when AI systems fail or are unavailable (Q1, Q2, Q9, Q10).<sup>3</sup>
6. Support adoption in resource-constrained and non-tech-hub settings  
Pair AI incentives with infrastructure and technical assistance for rural, small, safety-net, and other resource-limited organizations, so AI does not deepen the divide between large, well-resourced systems and everyone else (Q1, Q2, Q8, Q9).
7. Close the "model-to-practice" gap for federally funded AI.  
Encourage NIH and other HHS funders to support not only model development but also implementation, evaluation, and operations partnerships, so that fewer AI models end at publication and more are translated into tools that improve care in practice (Q1, Q2, Q8, Q10).<sup>4</sup>

<sup>1</sup> <https://www.jmir.org/2025/1/e64721>

<sup>2</sup> <https://www.jonesday.com/en/insights/2026/02/hhs-signals-policy-direction-to-accelerate-adoption-of-ai-in-clinical-care>

<sup>3</sup> <https://ojs.unsysdigital.com/index.php/ijas/article/view/10.21535%252Fijas.v3i2.916/0>,  
<https://pmc.ncbi.nlm.nih.gov/articles/PMC6389673/>

<sup>4</sup> <https://pmc.ncbi.nlm.nih.gov/articles/PMC7584400/>, <https://hsph.harvard.edu/exec-ed/news/implementation-gap-artificial-intelligence-help-patients-clinical-practices/>